



Fiona Stanley Fremantle Hospitals Group

Guideline

This policy is currently under review. It has been assessed by content experts and considered fit for purposes.

Inpatient admission to discharge guidelines and responsibilities

Reference #: FSFH-MENH-GUI-0011

Scope

Site	Service/Department/Unit	Disciplines
Fremantle Hospital Fiona Stanley Hospital Cockburn Health	Mental Health Service	Medical, Nursing, Allied Health, Administration

1. Introduction

This document outlines the roles and responsibilities of clinicians throughout the patient journey from referral to inpatient admission and discharge from the Mental Health Service.

2. Guideline

Refer to [Appendix 1](#)

3. Compliance/ Performance monitoring

The Service 5 Safety Quality and Risk Committee will monitor clinical incident data and near misses relating to clinical risk assessment and management, delays in access to care and transfer of care.

4. Related policy documents

- WA Health:
 - [Statewide Standardised Clinical Documentation for Mental Health Services \(MP 0155/21\)](#)
 - ['Triage to Discharge' Mental Health Framework for State-wide Standardised Clinical Documentation](#)
- SMHS:
 - [Care Coordination in mental health](#) (SMHS:13)
- FSFHG:
 - Admission and Bed Allocation (FSFHG Bed Management FSFHG GP 01)
 - Clinical Handover (FSFH-HW-POL-0035)
 - Clinical Documentation (FSFH-HW-POL-0039)
 - Clinical Risk Assessment and Management (FSFH-HW-POL-0017)
 - Post discharge follow up for mental health consumers (FSH-MENH-POL-0019)

5. Related standards

NSQHS

- Partnering with Consumers
- Comprehensive care
- Communicating for Safety

6. Other Related documents

- The Mental Health Act 2014

7. Authorisation

EXECUTIVE SPONSOR: Nurse Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Authorised By	Revision due
1	04/2015	Clinical Nurse	FH MHS Clinical Governance Committee	FH Policy & Legislative Compliance Committee	04/2018
2	09/2016	Clinical Nurse	FH MHS Clinical Governance Committee	FH Policy & Legislative Compliance Committee	09/2019
3	04/2021	CNS, Clinical Improvement	S5 Safety Quality Risk Committee	S5 Safety Quality Risk Committee (Single Speciality Process)	04/2025
3.1	12/2023	CNS, Clinical Improvement	Minor update no change to review date	S5 Safety Quality Risk Committee (Single Speciality Process)	04/2025
3.2	07/2024	S5 SQR LEAD	Minor update no change to review date	Service Director S5	04/2025
3.3	10/2025		Disclaimer added	FSFHG Policy	

8. Appendix

8.1. Appendix 1: Inpatient admission to discharge guidelines and responsibilities

	Referral	Admission	Inpatient	Discharge	Follow-up
Care Coordinator	If the Care Coordinator is the referring clinician, they will: - Facilitate the admission process by liaising with Admission Readmission Discharge Transfer (ARDT) Coordinator to identify bed availability. - Place patient on enterprise bed management system (EBM) with the priority. - Ensure the patient's needs are met prior to admission. - Review the patient daily whilst waiting for an inpatient bed (face to face if under MHA 2014) and document in DMR. Review risk and update risk assessment, prioritise if risk changes, informing treating psychiatrist, document in DMR. - Team leader/ CNS to update the ARDT Coordinator about patients on EBM prior to bed meeting at 08:50hrs. - Review - The ARDT Coordinator will liaise with the inpatient tea, and shift coordinator about referrals. ARDT Coordinator will accept/decline on EBM. - Ensure State-wide standardised clinical documentation (SSCD) is completed: - RAMP - MH Assessment form - Update TSDP	When an inpatient bed is available: - Care Coordinator/CTT or first on call (FoC) will be contacted either by the NUM or ARDT Coordinator informing of available bed. - Handover any issues warranting inclusion on the Treatment Support and Discharge Plan (TSDP) to the admitting nurse - Ensure Personal Support Person (PSP) is identified and documented in the patient's medical record - If the patient is under the MHA14, ensure all documentation is completed on PSOLIS and the PSP/nominated person is informed - Check for presence of Advanced Health Directive. - Planning for discharge to begin at the time of admission. When an inpatient bed is NOT available: - Liaise with Patient Flow to explore inpatients beds in other HSPs. - Consider presentation to FSH ED - Treating Dr to liaise with FSH ED consultant to discuss admission - Complete transport documentation if required to transfer the client to ED under the MHA 2014	- Provide a verbal handover where possible to the Primary Nurse / Shift Coordinator - Responsible for (or arranging proxy) attending: - Team Leader/CNS attends weekly FH multidisciplinary ward rounds. - In reach into FSH patient meetings - family meetings - interagency meetings - discharge and follow-up planning meetings - Continue engagement and active participation with patient throughout admission as required across the campuses. - Document engagement in the digital medical record (DMR) - Hand over significant clinical events to all relevant clinicians in a timely manner. - Reviews and updates TSDP with patient and Primary Nurse within 24 hours of admission. - Coordinate multidisciplinary services including external agencies as needed. - Participate in ongoing discharge planning. - Review phase of care - Notify the MDT if the patient is a current NDIS participant	- Coordinate ongoing care and treatment with the multidisciplinary team including frequency of follow up. - Make or arrange face to face contact with patient within 72 hours of discharge where possible. - Arrange post discharge face to face appointment with Community Treatment Team (CTT) with the patient and document in the DMR. .	- Coordinate follow-up care. - Review TSDP with patient. - Update phase of care, complete NOCC - Explore transport options for attendance to appointments. - Any patient transferring care from one community treating team to another within the Mental Health Service will have a non-medical care coordinator allocated for the transition period.

	Referral	Admission	Inpatient	Discharge	Follow-up
Referring clinician	Where the referring clinician is not the Care Coordinator.	- Complete full psychiatric assessment and risk assessment document on MH Assessment Form and RAMP and relevant MHA 2014 Forms. - Place patient on EBM and contact ARDT Coordinator - Once a bed is available the NUM/ARDT Coordinator/Patient Flow/HOOT contact referrer / patient / care co-ordinator to determine bed availability and inform ward of expected arrival. - If the patient is admitted from another site, medication charts, medical clearance and recent assessments are to be sent/ update DMR - Ensure the patient PSP/ nominated person is informed.			
Medical	- Face to face assessment of patient and DMR updated. - Decision made to admit patient, complete SSCD. - Liaise with GP or referrer, if appropriate - Place patient on EBM - Liaise with the ARDT Coordinator to identify if there is a bed available. - If no bed available liaise with Patient Flow. - Complete transport documentation if required.	When an inpatient bed is available: - ARDT Coordinator to review referral and liaise with the referrer if further information is required. - The referrer will be contacted either by the NUM or ARDT Coordinator informing where the available bed is. - Ensure MH Assessment form, Risk assessment and MHA 2014 forms are completed and up to date - Ensure - MH Assessment Form and Risk Assessment is completed and up to date - medication charts and agitation and arousal chart are completed - the patient is medically fit to be admitted to a MH ward. - Ensure the patient is medically cleared to be admitted to a MH ward. - If the patient is under the MHA 2014, ensure all documentation is completed on PSOLIS and the PSP is informed, and notifications are updated on PSOLIS. - Check for presence of Advanced Health Directive. - Planning for discharge to commence at the time of admission. - If patient < 18 yrs and admitted complete MHA 2014_s303 Segregation of Child from adult inpatients form and file a copy report in DMR, copy to parent guardian and	- Intern will complete physical examination and organic screen within 12 hours of admission – document on SSCD Physical Examination Form - Check for presence of Advance Health Directives. - Secure ward – review daily (am). - Reviews are to include Consultant or Registrar minimum of weekly. - Weekly Multidisciplinary Team (MDT) review. - Identify a Care Coordinator if there is none. - Provide a copy of any MHA 2014 forms to the patient and nominated person if there is one identified. - Inform the patient and family / carer of implications of the MHA 2014 and their rights under the act and update PSOLIS notifications - Ensure the TSDP is updated weekly post MDT and in collaboration with the patient and carers and ensure it is signed by the patient. Review phase of care. - Ensure that the patient has a copy of the TSDP - Initiate the Discharge Summary (NaCS) and update continually. - Discharge planning meeting to include the Care Coordinator and relevant stakeholders, including family and carers. - Ensure documentation of all reviews in the DMR - Discharge planning to continue, including predicted day of discharge (PDD). - Communicate the PDD to the patient at each review - Hand over significant clinical events to all clinicians including the Care Coordinator, in a timely manner - Liaise with GP. - Inform family and carer(s) of any increase in the patient's level of risk or any notifiable events and update DMR - Family meetings to be conducted at appropriate times during admission, and when critical events occur and record in DMR. - Document discharge and prescribe discharge medications	- Ensure discharge medications are charted. - Discharge medication chart to be completed 24 hours before discharge; 3 days prior in the case of Webster Packs. - Inpatient Registrar to ensure face to face appointment made to review patient within 7 days of discharge - After post discharge review, inpatient Registrar to arrange follow-up appointment for patient with community Registrar if that is part of the discharge plan. - Discharge summary (NaCS) to be completed on the day of discharge, with a copy to the patient. - Mental Health Assessment updated if required - Where the patient is discharged on Form 5A Community Treatment Order (CTO) this will be completed off the Form 6A - Inform CTT if MHA 2014 Form 6A is revoked and patient no longer on a CTO - Where a patient is accepted by a CTT from inpatient care,	- The inpatient intern is responsible for ensuring phone contact is made within 24hrs and documented as 'post discharge follow up' in PSOLIS - If the 24hr phone call is planned over the weekend or public holiday the intern will refer to ATT via E-referral who conduct the phone call. - Registrar to see patient as outpatient within 7 days of discharge and document as Service Event 'post discharge follow up' in PSOLIS. - Ongoing follow-up with outpatient Registrar/ Consultant if appropriate - Liaise with GP, clinic letters, scripts.

	Referral	Admission	Inpatient	Discharge	Follow-up
		the Chief Psychiatrist When there isn't an inpatient bed available: - Consider presentation to FSH ED - Liaise with FSH ED consultant to discuss admission - Liaise with Patient Flow to explore beds in alternative HSPs	on Medication Chart. - Provide supporting evidence for NDIS as required - Attend relevant interagency meetings	governance of clinical care will be transferred to the CTT on discharge from the ward	
Administration		- Enter patient admission and update patient details in PSOLIS/webPAS. - Confirm Medicare details and enter on system. - Identify if private patient admission - Prepare admission pack. - Confirm financial status. 08.00-20.00hrs enter referral in webPAS /PSOLIS. - Print patient labels. - Collate lists to send to intake meeting.	- Maintain inpatient file/medical record - Print patient labels as required. - Discuss Private Health Insurance status.	- Discharge summary (NaCS) mailed to patient's GP. - Discharge patient from webPAS/PSOLIS. - Collate patient's medical record, file loose reports and inpatient documents. - Complete checklist. - Ensure referral in PSOLIS if required	- Outpatient receptionist to confirm patient contact numbers and address. - Book 7 day post discharge follow-up appointments.
Inpatient	Acknowledge patient referral on EBM and pull patient when bed available	- Accept patient on EBM after liaison with the ward NUM/ARDT Coordinator. - Ensure ward is prepared to receive patient e.g. staffing, safety requirements, and interpreter if required. - Seek handover from Referrer. - Assess and document mental health status. - Complete nursing admission and assessment. - Ensure PSP are identified and documented in the patient's medical record - Check for presence of an Advanced Health Directives. - If the patient is declined for admission ARDT Coordinator to enter comment in EBM on reason for decline and inform the referrer.	- For new patients, the Primary Nurse becomes Care Coordinator until community Care Coordinator is allocated. - Confirm patient identification - armband and photo. - Orientate patient to the ward, including Patient Trust/Property, provide patient the patient information pack and ensure the carer is provided the Carer Information Booklet. - Commence TSDP on PSOLIS - Advise inpatient Registrar of patient arrival on ward. - Advise relevant clerical staff of admission. - Order patient meals noting any allergy - Complete admission NOCC - Nursing staff will complete weight, height, waist circumference, admission NOCC and monitor physical obs. - Conduct and record in DMR daily Mental State Examination and risk assessment - For active / current patients, ensure the community Care Coordinator is informed of the admission. - Ongoing liaison with medical, allied health and community Care Coordinator. - Nursing staff will attend ward rounds and hand over relevant information. - Ensure the TSDP is updated weekly post MDT and in collaboration with the patient and carers and ensure it is signed by the patient. - Ensure that the patient/carers has a copy of the TSDP - Hand over significant clinical events to all clinicians/care coordinator in a timely manner. - Inform family / carer(s) of significant events such as a change of ward or of leave status as appropriate - Confirm GP details and advise ward clerk.	- Discharge NOCC. - Liaise with medical staff. - Ensure patient has discharge medications and check discharge address and telephone number documented on the discharge envelope. - Provide patient with the white discharge envelope with a 7-day post discharge appointment if appropriate - Ensure patient has crisis numbers - Ensure community Care Coordinator is aware of discharge and any relevant stakeholders. - Ensure patient is aware of follow up appointments including the 24 hr phone call confirm the telephone number. - Check Patient Trust/Property sheet and return any goods to patient. - Discuss discharge transport options and facilitate as required. - Complete Discharge Checklist.	FSH: - After Hours CNM completes 7 day follow up for MHA - Team member for MHB and MBU completes 7 day follow up - Document as 'post discharge follow up' service event in PSOLIS Cockburn Health: - Team member for each ward completes 7 day follow up. - Document as 'post discharge follow up' in PSOLIS.

	Referral	Admission	Inpatient	Discharge	Follow-up
			- Obtain Depot Chart if relevant - If the patient is under the MHA 2014 ensure that they have a copy of the MHA 2014 form and their rights, and complete the notifications on PSOLIS - Ensure the PSP/nominated person is informed of the patients MHA 2014 status. - Coordinate and attend pre-discharge and follow-up planning meeting. - Inform family/carer/guardian of planned discharge.		
Allied Health	If patient is known, Allied Health will provide consultation to the inpatient team to ensure appropriate and smooth transition from community to inpatient setting	- Support the development of the TSDP - Where patient is known, OT will obtain clinical handover from previous OTs and stakeholders, and provide information to inpatient teams to ensure functional needs are appropriately met during admission - Where patient is known, CP will provide information to inpatient team to ensure smooth and effective transition into the Youth Unit - Conduct screening to identify necessary allied health input	- Conduct assessments and interventions relevant to the discipline and document in the DMR. - Identify complexities in management and any psychosocial complex needs that might act as barriers to recovery and discharge (all clinicians) and update DMR. - Contribute the TSDP to support patient's recovery - Ensure that any carers involved in the patient care have Carers Information Booklet. - In collaboration with the Care Coordinator arrange family meetings and stakeholders meeting where appropriate. - Participate in MDT meetings, multi-agency meetings, family meetings and discharge planning. - Pharmacist to perform Medication reconciliation as early as possible in the admission. - Hand over significant clinical events to all relevant clinicians in a timely manner and update in the DMR - Confirm financial status, whether on benefits. - Confirm NDIS status and complete referral to NDIS Hospital Liaison Officer (HLO) - Liaise with CTT i.e. Clinical Psychologist, to inform MDT inpatient treatment planning and role of CP. - CP to provide specialist consultation and advice to MDT in relation to difficult behaviours and provide input into behaviour management planning. - CP input if patient is referred for diagnostic assessment, risk management, psychological groups and 1:1 interventions. - Comprehensive Clinical Neuropsychological investigations for capacity assessment, differential diagnostic clarification and assist with treatment and discharge planning options - If patient is already known through previous admissions, CP role will be to continue treatment support, provide risk containment and if determined appropriate provide specific therapeutic interventions (e.g., trauma therapy) in consultation with community treating personnel. - Facilitate groups designed to develop skills and insight. - Ask patient to complete YES survey prior to discharge - Promote use of Safewards interventions - Support facilitation of patient morning meetings - Work with the NDIS HLO to establish if the patients NDIS plan will	FSH: - Youth Unit CP can provide transition support as an outpatient for six sessions post discharge whilst patient awaits commencement of community treatment. - CPs to be involved in assessment for and provision of Patient Controlled Crisis Admission Plans for future admissions SPECIFIC TO CP - Attend discharge planning meeting to discuss recommendations for ongoing psychological therapy with CP. - Meet with patient to discuss discharge including ongoing supports/treatment, anxiety related to discharge and relapse prevention. - Provide recommendations for ongoing community (psychological) treatment and assist patient to access Mental Health Care Plan (MHCP) via GP to access private CP. - Liaise with community MH team/case manager/CP to provide clinical handover including recommendations for ongoing psychological therapy. SPECIFIC TO OT: - Attend discharge planning meeting to discuss recommendations for ongoing Occupational Therapy interventions in the community. - Liaise with and provide	Complete 7 day follow up if allocated at point of discharge and document as 'post discharge follow up' in PSOLIS

	Referral	Admission	Inpatient	Discharge	Follow-up
			support their discharge and submit evidence if review is required • Provide orientation to the Comfort Room, calm down methods box and use of sensory interventions to support self-regulation • Coordinate visiting care providers (i.e. NDIS support workers) SPECIFIC TO OT: • Conduct initial assessment by gathering information regarding the consumer’s functioning, occupational performance, health management, social/physical living environment and consumer goals • Conduct functional, sensory and cognitive assessments to determine current level of functioning, identify strengths and barriers to recovery, provide recommendations about environmental and support needs required following discharge to maximise independence, health and wellbeing. • Provide aids and equipment for ward-use and/or issue equipment required on discharge. • Conduct home environment assessment if required to assess a consumer’s functioning within their living environment, review home environment, or issue pADL aids or equipment required for safe discharge. • Provide individualised interventions in self-care, productivity, leisure and psychosocial interventions to increase occupational participation, independence and support overall mental health recovery • Identify suitability and promote engagement in activity, skills and education-based group therapy as part of the multidisciplinary group therapy program • Support and promote consumer engagement in self-directed activity • Contribute to the FRAMP-CI and provide relevant intervention	clinical handover to community team, community OT, and other relevant stakeholders	